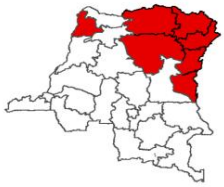
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Ministry of Public Health, Hygiene and Social Welfare
National Institute of Public Health
Public Health Emergency Operations Center
PRESIDENTIAL EBOLA TASK FORCE 17
MVE17 Incident Management System



Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°160/MVE-BDBV/21/10/2026

Reporting date: October 21, 2026 Publication date: October 22, 2026

HIGHLIGHTS (last 24 hours)

In total, 71 new confirmed cases and 41 deaths (23 community deaths and 18 intra-ETC deaths) of Ebola Virus Disease (EVD) cases have been reported in the provinces of Ituri (40), North Kivu (27) and Haut-Uélé (4). A total of 580 new samples were analyzed; 71 of these samples tested positive for the Bundibugyo virus, representing a positivity rate of 12.3%.
No new health zones have been affected in the last 24 hours.

ACCUMULATIVE CAS	ACCUMULATIVE CONFIRMED DEATHS	LETHALITY	PATIENTS IN ISOLATION/CTE	ACCUMULATIVE HEALED	TRACKING RATES CONTACTS (Of the Day)
9765	4727	48,4%	1091	2 618	84,8 %

Affected provinces and affected health zones

8 Provinces affected

Uele, Ituri, North Kivu, South Kivu, Tshopo, Lower Uele South Ubangi, and Nord Ubangi

66 Health Zones affected out of 178 (37.1%)

Summary of key points from the last 24 hours

71 **new** confirmed cases of Ebola virus disease (EVD) have been reported in three provinces: **Ituri (40 cases), North Kivu (27 cases)** and the **Haut-Uélé (4 cases).** Among these cases, **23 community deaths** were reported (13 in Ituri, 8 in North Kivu, and 2 in Haut-Uélé). In addition, **18 confirmed deaths** were recorded in Ebola Treatment Centers (14 in Ituri and 4 in North Kivu), bringing **the total number of confirmed deaths** reported during the period to 41.

o In addition, **32 patients (21 in Ituri and 11 in North Kivu)** were declared cured after obtaining two negative results in the control test for Ebola virus disease.

o As of October 21, 2026, the DRC has accumulated **9,765 confirmed cases, including 4,727 deaths,** representing a case fatality rate of **48.4** %. Ituri province remains the epicenter, accounting for **72.5%** of reported confirmed cases and **56.3%** new confirmed cases.

No new health zones have been affected in the last 24 hours. Thus, 66 out of 178 health zones (37.1%), spread across eight provinces, have been affected since the start of the epidemic. Ituri has 28 out of 36 affected health zones, followed by North Kivu (17/34), Haut-Uélé (7/13), Tshopo (7/23), Bas-Uélé (4/11), South Kivu (1/34), South Ubangi (1/16) and North Ubangi (1/11).

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Table 1. Distribution of confirmed cases and deaths by affected province, as of October 21, 2026.

Province	New confirmed cases (24h)	Confirmed cases	Deaths (confirmed)	Lethality	Affected health zones
Ituri	40	7080	3 298	46,6%	28/36 (77,8 %)
North Kivu	27	2163	1 210	55,9%	16/34 (47,1 %)
Haut-Uélé	4	425	185	43,5%	7/13 (53,8 %)
Boss	0	76	27	35,5%	7/23 (30,4 %)
South Kivu	0	3	1	33,3%	1/34 (2,9 %)
Bas-Uélé	0	13	4	30,8%	4/11 (27,3%)
Sud Ubangi	0	2	1	50,0%	1/16 (6,3%)
Nord Ubangi	0	3	1	33,3%	1/11()
Total	**71**	**9 765**	**4 727**	**48,4%**	**63/167 (37,7 %)**

Table 2. Distribution of confirmed cases and deaths by province and zone as of October 21, 2026

Province / Health Zone	Cumulative number			Current situation (24 hours)			
	Cas (n)	Deaths (n)	Lethality	New Case	Community deaths (Swab+)	Confirmed deaths within the CTE	Total confirmed deaths
Ituri	**7080**	**3298**	**46,6%**	**40**	**13**	**14**	**27**
Give	11	1	9,1%				0
Understand	17	6	35,3%	2	0	0	0
Ariwara	8	2	25,0%				0
Aungba	12	5	41,7%				0
Bamboo	216	48	22,2%				0
have	2	2	100,0%				0
Bunia	1923	649	33,7%	9	3	3	6
Ladies	41	10	24,4%				0
Drodro	15	7	46,7%				0
Fireworks	84	31	36,9%				0
Gethy	11	7	63,6%				0
Flounder	2	0	0,0%				0
Kilo	38	12	31,6%				0
Team	191	117	61,3%			1	1
Liter	315	158	50,2%				0
Logo	14	9	64,3%				0
Fight	36	9	25,0%				0
distributed	5	1	20,0%	1	0	0	0
Washing	77	33	42,9%	1	0	0	0
Darkness	380	201	52,9%	18	6	4	10

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Mangala	382	166	43,5%			1	1
Mongbwalu	717	346	48,3%				0
Nia -Nia	337	196	58,2%	3	3	0	3
Nizi	836	306	36,6%	4	1	2	3
Legumes	134	41	30,6%				0
Jungle	19	5	26,3%				0
Rwampara	1175	412	35,1%	2	0	3	3
Tchomia	82	25	30,5%				0
A valves	THAT	493	THAT				0
North Kivu	**2163**	**1210**	**55,9%**	**27**	**8**	**4**	**12**
Me	552	372	67,4%	9	5	3	8
Farm	24	8	33,3%				0
Butembo	323	190	58,8%	4	0	0	0
Ten	1	0	0,0%				0
Kalunguta	84	31	36,9%	3			0
Cut	651	398	61,1%	3	2	1	3
Source	3	2	66,7%				0
Kyondo	44	21	47,7%				0
Lubero	37	14	37,8%	1	0	0	0
Be careful.	86	31	36,0%	3	0	0	0
Alimbong	5	3	60,0%				0
Manguredjipa	16	5	31,3%				0
Masereka	39	13	33,3%				0
The museums	154	72	46,8%				0
Mutwanga	33	9	27,3%	2	0	0	0
Oicha	56	25	44,6%	2	1	0	1
Vuh	55	16	29,1%				0
Upper Uele	**425**	**185**	**43,5%**	**4**	**2**	**0**	**2**
Government Mangbetu	45	19	42,2%				0
Dumb	1	1	100,0%				0
Gombari	3	1	33,3%				0
Mathematics	108	48	44,4%				0
Lining	107	61	57,0%	1	1	0	1
Beans	6	3	50,0%				0
Ordinary	155	52	33,5%	3	1	0	1
Boss	**76**	**27**	**35,5%**	0	0	0	0
Bafwasende	18	3	16,7%				0
Kabondo	10	2	20,0%				0
Lubunga	1	0	0,0%				0
Maquis - Kisangani	33	14	42,4%				0
Mangobo	10	7	70,0%				0

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Boss	3	0	0,0%				0
Wanie-Arugula	1	1	100,0%				0
South Kivu	**3**	**1**	**33,3%**	0	0	0	0
Miti-Murhesa	3	1	33,3%				0
Bas Uélé	**13**	**4**	**30,8%**	0	0	0	0
Blind	1	1	100,0%				0
Viadana	6	1	16,7%				0
Claim	1	0	0,0%				0
Walk	5	2	40,0%				0
Sud Ubangi	**2**	**1**	**50,0%**	0	0	0	0
Hair	2	1	50,0%				0
Nord Ubangi	**3**	**1**	**33,3%**	0	0	0	0
You are logged in.	3	1	33,3%				
Total	**9765**	**4727**	**48,4%**	**71**	**23**	**18**	**41**

*These are the confirmed deaths that occurred in the various CTEs in the province which are being broken down to be classified by health zone.

NA : Not Applicable.

Table 3. Status of alerts notified by province for October 21, 2026

DPS	Alerts Received		Total alerts	Alerts verified and validated (suspected case)		Alerts verified and invalidated (no suspected cases)		Number of suspects investigated	Number of suspects investigated and transferred to CT/CTE (of the day)
	Living	Deceased		Living	Deceased	Living	Deceased		
Haut-Uélé	111	3	114	15	3	96	0	18	6
Bas Uélé	186	0	186	26	0	160	0	26	0
Ituri	1 154	76	1 230	160	68	962	6	216	132
North Kivu	1 477	63	1 540	138	59	1 339	4	197	116
Sud Ubangi	ND	ND	ND	ND	ND	ND	ND	ND	ND
Nord Ubangi	ND	ND	ND	ND	ND	ND	ND	ND	ND
Boss	140	0	140	12	0	128	0	12	1
Grand Total	**3 068**	**142**	**3 210**	**351**	**130**	**2 685**	**10**	**469**	**255**

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Figure 1. Evolution of cases by date of symptom onset over the last 21 days

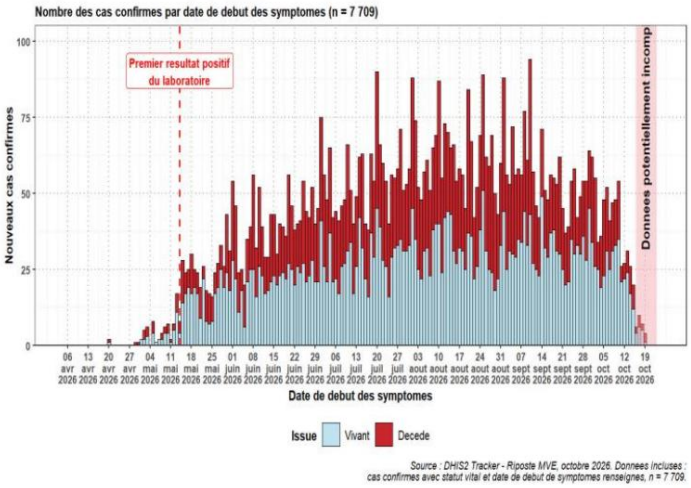
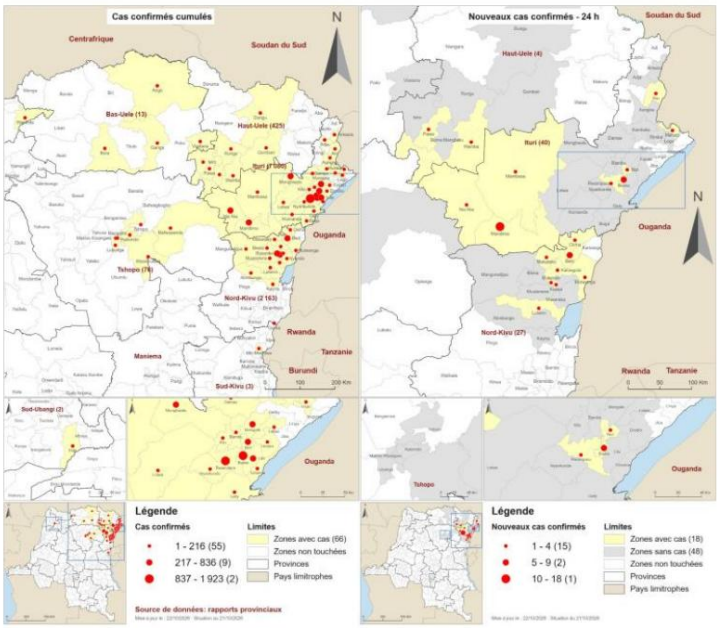


Figure 2. Spatial distribution of confirmed cases (24 hours and cumulative) by health zone.



1. COORDINATION

1.1. Main Actions

ÿ Support for response activities in the affected provinces through the technical pillars.

2. RESPONSE ACTIONS BY PILLAR

2.1. Epidemiological surveillance

2.1.1. Main Actions

ÿ As of October 21, 2026, 3,210 alerts had been reported in the five provinces that submitted their data, including 3,068 alerts of living individuals and 142 of deaths. Of these alerts, 481 (15.0%) were validated as suspected cases, and 2,695 (84.0%) were confirmed as suspected cases. Of the 481 confirmed suspected cases, 469 (97.5%) were investigated, with 255 (54.4%) transferred to COVID-19 treatment centers.

ÿ Of the 33,736 contacts under follow-up in the six provinces that submitted their data, 28,615 were seen in the last 24 hours, representing a follow-up rate of 84.8%, up from the previous day (79.7%) but below the required threshold of 95.0%. This rate reached 97.3% in South Ubangi (109/112), 95.5% in Bas-Uélé (714/748), 90.0% in Haut-Uélé (1,722/1,913), and 87.0% in Ituri (8,895/10,222). It remains insufficient in North Kivu (83.0%; 16,110/19,410) and Tshopo (80.0%; 1,065/1,331).

2.2. Update on checkpoint and point of entry (PoE/PoC) activities

2.2.1. Main actions

ÿ On October 21, 2026, seven (7) alerts were notified to the PoE/PoC: 4 in Ituri, 2 in North Kivu, and 1 in South Kivu. In Ituri, 3 alerts involving live individuals were confirmed (Makoko 2, Base Yedi, and Biakato Mayi), including 2 travelers who refused transfer to the CTE, and a body was secured in Mudzi Bala (Bunia). In North Kivu, the alert from the Maboya PoC (Kalunguta) was confirmed and referred to the Tinge Hospital. In South Kivu, a body repatriated from Burundi was notified to the Kavimvira PoE.

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Table 4. Status of key PoE/PoC indicators by province, as of October 21, 2026

Indicators	Ituri	North Kivu	South Kivu	Boss	Haut-Uélé	Global
Report completeness	98,4% (61/62) 95,2% (20/21)		55,6% (5/9) 100,0% (18/18) 80,0% (8/10)			93,3% (112/120)
% of functional strategic PoE/PoC	100%	100%	ND	100%	80%	ND
Number of people who have passed PoE/PoC	158 965	94 238	10 163	20 521	23 086	306 973
% of travelers screened	97,9%	98,4%	98,8%	99,3%	98,6%	98,2%
% of travelers who washed their hands	97,5%	98,4%	98,8%	99,3%	99,0%	98,0%
% of travelers made aware	99,6%	98,4%	100,0%	100,0%	100,0%	99,3%
Number of alerts notified	4	2	1	0	0	7
Number of validated live alerts (cas suspects)	3	1	0	0	0	4
Number of bodies recovered today	1	0	1	0	0	2
% of suspected cases transferred to CTE/CT	33,3%	50,0%	THAT	THAT	THAT	41,6%
% of swabs/ lifeless body secured	100,0%	THAT	100,0%	THAT	THAT	100,0%
% of suspected cases investigated within 4 h	50,0%	100,0%	100,0%	THAT	THAT	75,0%

2.2.2. Challenges

In North Kivu, the Butuhe PoC has remained closed since September 26, 2026. PoC alerts are not being addressed promptly, and there is a lack of IPC supplies, PPE, and a dedicated ambulance. In Ituri, two out of three suspected cases refused transfer, and the Avakubi PoC remains non-functional due to insecurity. In South Kivu, the completeness rate remains low (55.6%), and PoEs lack isolation facilities. In Haut-Uélé, two out of ten PoCs did not report due to network connectivity issues. In Bas-Uélé, five out of twenty PoCs did not report; training, motivation, water, mobility, and digitalization are lacking.

2.3. Laboratory

2.3.1. Main actions

ÿ Ituri: 40 new positive results (27 living and 13 dead) out of 272 new samples analyzed (positivity: 14.7%).

ÿ North Kivu: 27 new positive results (19 living and 8 dead) out of 278 new samples analyzed in Beni and Butembo (positivity: 9.7%).

ÿ Haut-Uélé: 4 new positive results (2 living and 2 dead) out of 23 new samples analyzed (positivity: 17.4%).

ÿ Tshopo: 5 samples received and analyzed (5 live) the result was negative

ÿ Bas Uélé: 2 samples received and analyzed (2 living) the result was negative.

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2.4. Infection Prevention and Control (IPC/EDS)
2.4.1. Main actions

ÿ **Ituri:** 29 out of 46 rings were opened (63.0%), 47 out of 50 households were decontaminated (94.0%), and 8 out of 8 health facilities were decontaminated (100.0%). Of 87 EDS alerts, only 51 EDS were carried out (58.6%), and 50 out of 70 bodies were swabed (71.4%).

ÿ **Tshopo:** 16 out of 16 ESS (Emergency Health Services) were monitored in 6 health zones and 201 service providers were briefed; the Canana (case no. 75) and Yabiso (Kabondo) health centers were decontaminated and 4 replacement mattresses were distributed to 3 households. No DHS (Digital Health Survey) was conducted (6 postponements). **Haut-Uélé:** 9 out of 9 households were decontaminated in Boma Mangbetu, 57 service providers were briefed and 2 DHS were conducted in Wamba (1 body found).

ÿ **North Kivu:** 29 out of 51 rings were opened (56.9%), 21 out of 26 health centers were decontaminated (80.8%), and 11 out of 33 households (33.3%). Nine infection prevention and control (IPC)/water, sanitation, and hygiene (WASH) assessments were conducted in nine health centers (average score of 33.5%). Fourteen schools were decontaminated in Butembo, 27 schools received kits, and 31 public places were decontaminated. Of 76 death and health alerts received, 64 death and health assessments were carried out, and 64 bodies were swabed.

ÿ **Bas-Uélé:** 13 out of 13 ESSs were followed in 3 ZS (Buta, Ganga and Viadana), 83 out of 83 providers and 16 out of 16 hygienists were briefed and 2 scorecard assessments were carried out in Buta (CS du Congo 30% and CS Makala 63%).

2.4.2. Challenges

ÿ In Ituri, 17 out of 46 rings remain to be opened, 3 out of 50 households have not been decontaminated, 23 EDS failures have been recorded, 20 out of 70 bodies (28.6%) could not be swabed and 13 EDS are postponed to the next day.

In North Kivu, 22 out of 51 screening areas remain to be opened (Butembo, Mabalako), 22 out of 33 households have not been decontaminated, no households have received kits (0/11), and 45 mass casualty incidents (MSIs) have been postponed. In Tshopo, 4 bodies could not be swabed, and stocks of infection prevention and control (IPC) supplies are critical (25 kg of chlorine out of the 1,800 kg required). In Haut-Uélé, 1 body could not be swabed in Wamba (due to resistance). In Bas-Uélé, no MSIs have been carried out, and 16 out of 20 priority health facilities (HSFs) do not have functional triage.

2.5. Holistic care
2.5.1. Main actions

ÿ In Ituri, 151 new admissions (all suspected cases) were recorded, compared to 104 discharges, including 21 recoveries and 25 deaths (14 confirmed). Thus, 499 patients are hospitalized (178 confirmed and 321 suspected), representing an occupancy rate of 48%, including 57 critically ill patients; 474 out of 492 patients (96.3%) received hot meals.

In North Kivu, 57 new admissions (3 confirmed and 54 suspected) were recorded, compared to 77 discharges, including 11 recoveries, 7 discharges without medical advice, and 4 confirmed deaths. A total of 468 patients are currently receiving care (161 confirmed and 307 suspected), including 59 children under 5 years old and 6 critically ill patients, 137 of whom are outside of Ebola Treatment Centers (ETCs), for an overall occupancy rate of 80.7% (331/410 ETCs). 24 ambulance trips were successfully completed (12 out of 14 ambulances used).

ÿ In Tshopo, 4 new admissions (1 confirmed and 3 suspected) versus 1 discharge (non-case); 9 Patients are hospitalized in 36 beds, representing an occupancy rate of 25.0%.

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ÿ In Bas-Uélé, 23 new admissions of suspected cases were recorded and isolated in the ESS against 1 discharge (non-case in Bili), bringing the number of patients in isolation to 86; the 3 beds for confirmed cases are occupied (100.0%).

2.5.2. Challenges

ÿ In Ituri, blood supplies remain insufficient, 18 patients in Mandima have not received food, and the Accommodation capacity remains low in Kilo, Lolwa, Mandima and Biakato.

ÿ In North Kivu, 90.1% of the beds for suspected cases are occupied (173/192), 137 out of 468 patients (29.3%) are being treated outside of CTE/CT and the completeness of the data is only 79% (Kalunguta, Mabalako and Lubero missing).

ÿ In Tshopo, 5 confirmed cases remain wandering in the community (4 in Bafwasende and the case n°53).

ÿ In Bas-Uélé, the delay in setting up the standardized CTE at the HGR in Buta, the isolation of 86 suspects in ESS without nutritional support, the absence of an ambulance and the delay in psychosocial support for the 3 confirmed cases remain major challenges.

2.6. Vaccination 2.7.

Main actions

ÿ In Ituri, vaccination of people living with HIV (PLHIV) using Ervebo and research on the efficacy of the BDBV vaccine are continuing, bringing the cumulative total to 1,311 people vaccinated (770 men and 541 women, including 15 pregnant women and 33 breastfeeding women) with 28 reported adverse events following immunization (AEFIs) (26 non-severe and 2 severe); the Partners, EBO-PEP, and Oracle trials are ongoing, and the TUMAINI project teams are currently being briefed. In Bas-Uélé, the compassionate vaccination support team has arrived in the Ango health zone, and coverage has reached 11.4% of the target (1,500/13,202). Data from North Kivu are not yet available.

2.7.1. Challenges

ÿ In Ituri, the target population for TPL and PPL remains to be redefined, and two serious AEFI cases have been reported; in Bas-Uélé, the CCCs are not operational, and vaccination has not yet begun in Ango (0/2,247) or Bili (0/1,067); in Haut-Uélé, the community is questioning the exclusive targeting of PPL

2.7.2. Continuity of Care

2.7.3. Main actions

ÿ In North Kivu, the baseline CSSE assessment of HGR Katwa reveals the absence of a simplified CSSE plan, triage not respecting the circuit, insufficient PPE and blood products, the absence of an ambulance and a contingency plan for absenteeism; out of 178 available providers (all trained in PCI), 18 are absent and 3 are exposed or in quarantine.

ÿ In Ituri, the recovery plan for Komanda General Hospital was developed and IT staff were trained in the mass administration of antimalarial drugs.

ÿ In Bas-Uélé, the monitoring visit of the construction work at the Buta CTE was carried out with with the support of the WHO.

ÿ In Tshopo, the evaluation of the Lotoyano polyclinic reveals ineffective triage and 6 out of 12 agents vaccinated (50.0%).

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2.7.4. Challenges

ÿ Poor functionality of triage at Komanda General Hospital (Ituri); absence of CSSE dashboard at Katwa General Hospital (North Kivu); decline in IMCI and deliveries in Tshopo.

2.8. Risk Communication and Community Engagement (RCCE)

2.8.1. Main actions

In Ituri, 16,594 home visits reached 65,154 people, 1,156 educational talks reached 18,930 people, 192 mass awareness campaigns reached 14,213 people, and 7 community debates were organized for 407 people. A total of 439 community alerts, including 135 deaths, were reported; 243 out of 272 feedback requests (89.3%) and 3 out of 13 incidents (23.1%) were resolved within 48 hours. The CREC facilitated the pre-listing of 363 contacts, the completion of 12 out of 13 EDS (Emergency Health Diagnosis) tests, and the transfer of 35 out of 40 suspected cases; the completeness of CREC reports remains low (11/28 health zones, 39.3%).

In North Kivu, 11,915 home visits reached 32,415 people, 229 educational talks were organized for individuals, and 64 mass communications reached 7,170 people; 440 community alerts were notified out of 650 expected (67.7%), 29 feedback messages were received out of 37 (78.4%), and 2 out of 2 incidents were resolved within 48 hours. Infodemic management training was organized in Beni for 8 health zone hotspots.

In Tshopo, 13 mass awareness campaigns reached 105,403 people and 4 talks reached 178 people; 44 community alerts were reported and 3 out of 3 feedback points were resolved. In Haut-Uélé, 315 home visits reached 1,890 people and 7 mass awareness campaigns reached 7,853 people; 75 community alerts were reported. In Bas-Uélé, 332 home visits reached 955 people; 7 community alerts were reported.

2.9. Mental Health and Psychosocial Support (MHPSS)

2.9.1. Main actions

ÿ In Ituri, 19 out of 37 confirmed cases (51.4%) benefited from SMSPS interventions, 88 out of 157 confirmed cases and 137 out of 321 suspected cases under follow-up were taken care of, 9 out of 13 separated and orphaned children (69.2%) were accompanied, 75 children are being followed up, 9 out of 9 children are in daycare, 9 out of 21 have recovered and 65 results have been announced, including 19 positive ones.

ÿ In North Kivu, 21 out of 29 confirmed cases (72.4%) benefited from SMSPS interventions, 136 confirmed and 115 suspected cases are being monitored, 37 out of 40 separated and orphaned children households (92.5%) were supported, 28 out of 36 children (77.8%) were monitored in the community, 6 out of 6 children are in daycare, 9 recovered and reintegrated and 123 results announced, including 29 positive; 426 people benefited from psychoeducation sessions.

ÿ In Tshopo, the newly confirmed case and all 6 new suspects were taken into care, and 365 people received psychoeducation. In Bas-Uélé, 1 new suspect received support.

2.9.2. Challenges

ÿ In Ituri, only 15 of the 28 affected health zones are covered (53.6%) and 18 of the last 37 confirmed cases (48.6%) have not yet been taken care of, notably in Mandima (4/15).

ÿ In North Kivu, SMSPS coverage remains low around the latest confirmed cases in Biena, Manguredjipa, Kayna, Lubero, Masereka, Kalunguta, Mabalako, Mutwanga and Vuhovi, 10 affected health zones do not have a nursery and reintegration kits for the recovered are lacking; in Bas-Uélé, psychosocial support for the 3 confirmed cases is still lagging behind.

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2.10. Logistics
2.10.1. Main actions

ÿ In North Kivu, 3 tents (UNICEF), 350 bottles of gel and 800 gowns were received and the health zones of Butembo, Katwa and Oicha were supplied with inputs for IPC, EDS and PEC; in Tshopo, 1 4x4 vehicle, 35 motorcycles and an ambulance were received; in Bas-Uélé, the construction of the Buta CTE continues, but the diesel stock is exhausted.

2.11. Security

2.11.1. Main actions

ÿ In Ituri, insecurity persists in 17 health districts (AS) in Mambasa and Mandima. In North Kivu, the health assessments (EDS) of 4 confirmed cases and the decontamination of 2 health centers (ESS) and one household have been secured. In Tshopo, threats against teams persist in Bafwasende. In Haut-Uélé, reluctance has been reported in Rungu.

2.12. PSEA (Prevention of Sexual Exploitation and Abuse)
2.12.1. Main actions

ÿ In North Kivu, 126 people were sensitized on the PSEAH and reporting to 495555/151 in the health zones of Katwa (16), Beni (94, with the support of WHO and DoS) and Mutwanga
(16 IT); in Haut-Uélé, 94 participants were sensitized at the CS Tuluba (Isiro), pregnant women at the CM VDO and PPL, nurses and cooks of the response in Wamba, where 8 codes of good conduct were signed

2.12.2. Challenges

ÿ At the provincial level, coverage of affected areas remains low: only 3 out of 34 health zones (8.8%) know the reporting mechanism in North Kivu and 6 out of 13 in Haut-Uélé; in Haut-Uélé, none of the 14 partners report their PSEA activities, awareness tools are only available in Swahili even though this language is only spoken in 3 health zones and the pillar lacks computer kits.

MESSAGES CLES

Over the past 24 hours, 469 out of 481 suspected cases (97.5%) have been investigated in the five provinces that submitted their data. However, 12 suspected cases remain uninvestigated in Ituri (8 in Rwampara, 7 in Nyankunde, and 2 in Bunia, with a negative value of -5 in Nia-Nia that needs correction), none of the 26 confirmed suspects in Bas-Uélé have been transferred to a treatment center/emergency care center, and only 1 of the 12 confirmed suspects in Tshopo has been transferred. Furthermore, the contact tracing rate is improving, rising from 79.7% the previous day to 84.8%, but remains below the required threshold of 95.0%, with a critical situation in Lita (32.5%), Kalunguta (54.7%), and Nia-Nia (59.6%). North Kivu recorded 27 new cases, its highest daily total in recent years, and Haut-Uélé reported 4 new cases (Wamba and Pawa) after a day without any. This situation necessitates strengthened epidemiological surveillance and contact tracing to limit the risk of community transmission.

Maintain a level vigilance regarding epidemiological surveillance aspects, consolidate actions.

TIMELINE OF KEY EVENTS

o From April 24 to May 15, 2026, the epidemic progressed from the detection of the first suspected cases to the biological confirmation of the Ebola Bundibugyo virus, leading to the official declaration of the 17th Ebola Virus Disease epidemic in the Democratic Republic of Congo.

o Between May 17 and 18, 2026, the national authorities, with the support of partners, strengthened the response by declaring a public health emergency of national and then continental scope, accompanied by the mobilization of funds.

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